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Oral Sadism

and the Vegetarian

Personality

Readings From the Journal of Polymorphous Perversity®

edited by

Glenn C. Ellenbogen, Ph.D.

[image file=image_rsrc5H6.jpg] A STONESONG PRESS BOOK

A Modern Day Psychoanalytic Fable

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To Ilene—my partner in laughter, love, and play.

Contents

Foreword

Judd Marmor, M.D.

Preface

Acknowledgments

1. Psychotherapy

The Etiology and Treatment of Childhood

Jordan W. Smoller

Psychotherapy of the Dead

Samuel E. Menahem, Ph.D.

Psychotherapy of the Dead Revisited: Biological Approaches to the
Treatment of a Difficult Patient Population

Terrei L. Templeman, Ph.D.

Developing Insight in Intensive-Extensive Psychodynamic Psychotherapy: A
Case Study

Don Yutzler, Ph.D.

The Effective Ingredient in Psychotherapy: An Alternative to the
Spontaneous Remission Hypothesis

Ann V. Deaton, Ph.D., and Mary Ellen Olbrisch, Ph.D.

A Proposal for Eliminating the Chronic Shortage of Mental Health Service
Providers: Hounds as Humanists

Wayne R. Bartz, Ph.D., and Roger E. Vogler, Ph.D.

A Unifying Theory of the Psychotherapies: Advice Therapy

John W. Zuboy, Ed.D.

Cognitive-Behavioral Treatment of Chronic Flatulence

Perry Stalsis, Ph.D.

A Great Moment in Gestalt Psychotherapy

Kirk E. Farnsworth, Ph.D.

Behavioral Strategies for Coping with Urban Crime: Vigilante Effectiveness Training (V.E.T.)

Michael F. Shaughnessy, Ph.D.

Thanatotherapy: A One-Session Approach to Brief Psychotherapy

Kathleen M. Donald, Ph.D., and Bruce E. Wampold, PhD.

2. Psychoanalysis

The Nasal Complex and Other Recent Advances in Psychoanalytic Theory

Perry Stalsis, Ph.D.

A Modern Day Psychoanalytic Fable

Alma H. Bond, Ph.D.

A Modern Day Psychoanalytic Fable: II. Goldi-Locks and the Three Bares

Alma H. Bond, Ph.D.

3. Psychodiagnostics

Oral Sadism and the Vegetarian Personality

Glenn C. Ellenbogen, Ph.D.

A Brief Report of a Psychodiagnostic System for Mental Health Clinic Patients: Parking by Diagnosis

Leon J. Schofield, Jr., Ph.D.

A Brief Report of a Psychodiagnostic System for Mental Health Clinic Patients: Diagnosis by Parking

John B. Pittenger, Ph.D.

The Digital Diagnosis and Treatment Finder: If s as Easy as ABC

Kathleen M. Donald, Ph.D.

Clinical Notes on “The Case of the Little Prince”

David Forbes, PhD.

A Quick-Screening Test of Mental Functions

Walter A. Kuciej, R.N.

On the Differential Diagnosis and Treatment of Mental Afflictions

Increase Mather and Cotton Mather

4. Psychological Testing

Nicholas Claus: A Case Study in Psychometrics

E. M. Bard, Ph.D.

The MMPI (Revised): The Midlife Maladaption Prognosis Inventory

Kathleen M. Donald, PhD.

The Minnesota Multiphasic Personality Inventory (MMPI) Revisited: 43 Years and 24 Test-Items Later

Robert D. Friedberg, MA.

A Rapid Screening Test for Neuropsychological Function

Robert S. Hoffman, M.D.

The Scale of Mental Abilities Requiring Thinking Somewhat (S.M.A.R.T.S.):
I. Test Construction, Normative Data, Administration, Test Items, and
Scoring

Glenn C. Ellenbogen, Ph.D.

5. Clinical Psychology

New Improved Delusions

Robert S. Hoffman, M.D.

A Brief Report on Clinical Aspects of Procrastination: Better Late than Never

Kathy Alberding, M.S. W., David Antonuccio, Ph.D., and Blake H. Teaman,
Ph.D.

Burdenism

Michael J. O'Connell, Ph.D.

Sexuality Survey

Stephen D. Fabick, Ed.D.

6. Educational Psychology and Education

An Experimental Investigation of Bad Karma and Its Relationship to the
Grades of College Students: Schwartz's F.A.K.E.R. Syndrome

Martin D. Schwartz, Ph.D.

The Influence of Tenure on the Productivity of Faculty in Higher Education

Edward A. Polloway, Ed.D.

Didactosemiology: A System for Covert Communication in Lectures,

Seminars, and Rounds

Robert S. Hoffman, M.D.

A Comprehensive Exam for Students in Introductory Psychology

J. Randall Price, Ph.D.

Practice Questions for the Psychology Licensing Exam

Adrianne Aron, Ph.D.

Final Impotence: Reasons Why I Missed the Test

Lillian M. Range, Ph.D., Morgan Banks, MA., and Timothy Leonberger, M.S.

Psychology Graduate School Interviews: Tips for the Applicant

Robert D. Friedberg, M.A.

7. Industrial/Organizational Psychology

A Validation Procedure for Telephone Linepersons: Implications for Sex Discrimination

Duane Dove, Ph.D.

8. Experimental Psychology

The Issue of No-Show Subjects: A Failure to Replicate Non-Data on Non-Volunteer No-Shows

Arnie Cann, Ph.D., Lawrence Calhoun, Ph.D., Ignatius Toner, Ph.D., Gary Long, Ph.D., and Margaret Hagan

Recurrence of the Déjà Vu Phenomenon Among Professional Psychologists

Larry Pate, Ph.D., and Warren Walker

Psychobotany: A Budding Field of Psychological Study

Stanley Messer, Ph.D., and Mickey Clampit, Ph.D.

Cancer and Tobacco: A Bum Rat

Jack L. Nasar, Ph.D., and Nick Ingoglia, Ph.D.

Recurrence of the Déjà Vu Phenomenon Among Professional Psychologists

Larry Pate, Ph.D., and Warren Walker

9. Developmental/Child Psychology

Toys for Tots: Recommendations by Psychological Experts

W. Scott Terry, Ph.D., Lawrence G. Calhoun, Ph.D., and Arnie Cann, Ph.D.

Perceptual Preference in Homo Professoria: No News Is Bad News

R.E.M. Kleinberg

10. Statistics

Mode to Stats

Amy Katz

A Statistically Significant Dinner at the Hotelling Vancouver

James Frankisk, Georgia Tiedemann, M.A., Pat Manly, M.A., and Ken Reesor, M.A.

11. History and Systems of Psychology

Prescriptions for Fame in the History of Psychology

W. Scott Terry, Ph.D.

12. Psychology Journals

[illegible]

Ernst von Krankmann, PhD., and Ilene Bernstein

the journal of underachievement

Ernst von Krankmann, Ph.D., and Ilene Bernstein

13. Book Reviews

A Book Review—Buddha Meets Kohut: Western Civilization as a Transitional Object

Reviewed by Tom Greening, Ph.D.

A Book Review—Pathways to Consciousness: Hashish, Spinach Quiche, and Rajneesh

Reviewed by Tom Greening, Ph.D.

14. Contemporary Issues in Psychology

Collaborative Research and Publication: An Experimental Investigation of the Dynamics Underlying the Trend Toward Multiple Authorship in Scholarly Psychological Publications

Edward A. Polloway, Ed.D., Thomas A. Looney, PhD., G Kenneth West, PhD., Rennte N. Motroopin, Ph.D., J. David Smith Ed.D., M.E. Gordon, Ed.D., Riccumulur Evita, Ph.D., Thomas W. Decker, Ph.D., Michael H. Epstein, Ed.D., Douglas Cullinan, Ed.D., James W. Patton, Ed.D., John T. McClure, M.A., Peter D.L. Warren, PhD., and Carl R. Smith, PhD.

Ethical Principles of Psychologists: An Update

Joel Herscovitch, PhD.

A Grammatical Overview of Medical Records: The Write Stuff

Corey D. Fox, PhD.

Foreword

“A little nonsense now and then
is relished by the wisest men”

Anonymous

I feel a particular pleasure in being asked to write a foreword to this delightful volume, not only because it allows me to honor it, but also because it gives me an opportunity to retrieve (from the wastebasket) an important early contribution of mine that never saw the light of day.

It was a “modest proposal” for a new approach to psychotherapy, based on the fact that all mammalian neonates, including those of human beings, manifest a prepotent need for mother’s milk, or its equivalent. This powerful oral craving persists throughout infancy, and failure to gratify it can, as we know, result in serious physiological and psychological consequences for the developing child.

Proceeding from this base, it seemed reasonable to assume that the reparation of such consequences could be facilitated by employing a richer-than-average form of milk equivalent, such as cream. The working hypothesis for this proposed treatment, therefore, was that if cream were provided, under appropriate circumstances, to neurotic subjects who had suffered severe oral deprivation in their early developmental years, it would provide a corrective therapeutic experience of great value.

The proposed methodology was to be as follows: A properly selected group of such patients would be placed in oversized infant cribs and administered a pint of cream in large nursing bottles three times daily. Supplemental infant food would be added as needed. Significant clinical improvement would be anticipated within three to six months.

Unfortunately, the Journal of Polymorphous Perversity had not yet begun publication at the time, and I was unable to obtain an appropriate grant to test

my hypothesis. Thus, my chance to have my name forever linked to a unique and original form of psychotherapy was lost. I would have called it—modestly—“Marmor’s primal cream therapy.”

On a more serious note, many years ago, in 1953, I wrote an article entitled “The Feeling of Superiority: An Occupational Hazard in the Practice of Psychotherapy,” in which I was concerned, among other things, about the danger of unconscious arrogance and grandiosity developing in psychotherapists because so much of their daily lives are spent with patients whose transferences lead them to overestimate the wisdom and capacities of their therapists. I added: “[but] it is not only the reactions of his patients that tend to foster ‘god-like’ feelings in the psychotherapist. The attitude of the lay public is equally significant in this regard.... The psychotherapist has become the shaman of our society, the all-seeing father with the Cyclopean eye. He is endowed with god-like perceptiveness ... he is either Deity or Devil, but rarely is he portrayed as he really is—a person with special training and ability, with human strengths and human frailties.”

I know of no better antidote to this occupational hazard than the ability to laugh at one’s self, and have the pomposities of jargon in which we tend at times to clothe ourselves—obfuscations that are often mistaken for profundities—deflated. This volume does this superbly—taking on and spoofing, one by one, the pretentious and ponderous pontifications that are often indulged in by various professionals in our field. They are all included here—psychotherapists, psychoanalysts, psychodiagnosticians, psychological testers, clinical psychologists, educational psychologists, industrial psychologists, statisticians, psycho-historians, professional journalists, and book reviewers.

No one who reads the essays in this book will ever again be taken in by the affectations of pedants in our field—and so much the better for all of us! I commend these essays to you with laughter and affection. Read and enjoy!

Judd Marmor, M.D.

Preface

The beginnings of Psychology as a “serious” science can be traced back to Wilhelm Wundt who, in 1879, established the first psychological research laboratory. Almost 100 years later, when I entered graduate school, Psychology had become a very serious science—too serious, I thought. I had no way of knowing, then, that I was not alone in thinking as I did. In contrast to all of the serious works that were being written in Psychology, I began writing humorous and satirical articles spoofing Psychology, continuing my writing throughout graduate school and beyond.

In 1980, I founded Wry-Bred Press, a small publishing company devoted to producing and distributing humorous spoofs of Psychology. The first works, which I authored, were humorous “monographs,” published under the banner of a fictitious periodical—the Journal of Polymorphous Perversity. As these monographs slowly made their way into the hands of psychologists and other mental health professionals over the next few years, I began to receive letters saying, “Your journal is great! Psychologists take themselves and Psychology too seriously. This is just what the field needs. Keep up the good work.” In addition, I began to receive submissions “for consideration by your journal.” I found more than just a little bit of humor in this situation—receiving humorous pieces for consideration by a journal that did not really exist. Clearly, I was being given the message that there was a real need for a forum whereby psychologists, like myself, could publish humorous and satirical pieces spoofing Psychology.

So, in the summer of 1983, I began to lay the groundwork for a real humorous and satirical journal of Psychology, not surprisingly to be called the Journal of Polymorphous Perversity. On January 2, 1984, the first issue of the Journal of Polymorphous Perversity was released, featuring “Psychotherapy of the Dead” as the lead article.

The articles comprising this anthology were drawn from the first three volumes of the Journal of Polymorphous Perversity and from the monograph series. In the three years of its existence, the Journal of Polymorphous Perversity has made quite a bit of progress in the difficult task of chipping

away at the seriousness of the members of the psychological community. This progress is reflected, for instance, in the fact that more and more state and regional psychological associations have sought permission to reprint the Journal of Polymorphous Perversity articles in the official magazines or newsletters of their organizations. But, perhaps one of the most salient examples of the Journal of Polymorphous Perversity's impact upon the field rests right in your hands—this anthology, itself, which Brunner/Mazel, Inc., a specialty publisher of serious scholarly and professional psychology and psychiatry texts, has chosen to publish! It does, indeed, provide me with hope that the Journal of Polymorphous Perversity will continue to make inroads in helping psychologists to be less serious and to take humor more seriously.

New York, New York Glenn C. Ellenbogen

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I would like to gratefully acknowledge the help of the following Journal of Polymorphous Perversity associate editors, each of whom was kind enough to review the many manuscripts that I forwarded to them for review when the topic touched upon their specialty area:

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The decision as to what constitutes “good” humor involves a very subjective process. Thus, the associate editors were rarely in agreement, either among themselves or with me, about which articles merited publication. The responsibility for making the final selections for inclusion in both the regular

journal issues and this anthology was ultimately mine and mine alone. The presence of any given article in this anthology should not be construed as reflecting the endorsement of the associate editors.

I would like to thank Dr. Martin Obler, who encouraged me in the development of my humorous perspective on life.

Lastly, I would like to thank the many authors whose works appear in this anthology.

Oral Sadism

and the Vegetarian

Personality

Readings From the Journal of Polymorphous Perversity®

1

Psychotherapy

The Etiology and Treatment of

Childhood 1,2

Jordan W. Smoller

University of Pennsylvania

Childhood is a syndrome which has only recently begun to receive serious attention from clinicians. The syndrome itself, however, is not at all recent. As early as the 8th century, the Persian historian Kidnom made reference to “short, noisy creatures,” who may well have been what we now call “children.” The treatment of children, however, was unknown until this century, when so-called “child psychologists” and “child psychiatrists” became common. Despite this history of clinical neglect, it has been estimated that well over half of all Americans alive today have experienced childhood directly (Suess, 1983). In fact, the actual numbers are probably

much higher, since these data are based on self-reports which may be subject to social desirability biases and retrospective distortion.

The growing acceptance of childhood as a distinct phenomenon is reflected in the proposed inclusion of the syndrome in the upcoming Diagnostic and Statistical Manual of Mental Disorders, 4th Edition, or DSM-IV, of the American Psychiatric Association (1985). Clinicians are still in disagreement about the significant clinical features of childhood, but the proposed DSM-IV will almost certainly include the following core features:

1. Congenital onset
2. Dwarfism
3. Emotional lability and immaturity
4. Knowledge deficits
5. Legume anorexia

Clinical Features of Childhood

Although the focus of this paper is on the efficacy of conventional treatment of childhood, the five clinical markers mentioned above merit further discussion for those unfamiliar with this patient population.

Congenital Onset

In one of the few existing literature reviews on childhood, Temple-Black (1982) has noted that childhood is almost always present at birth, although it may go undetected for years or even remain subclinical indefinitely. This observation has led some investigators to speculate on a biological contribution to childhood. As one psychologist has put it, "we may soon be in a position to distinguish organic childhood from functional childhood" (Rogers, 1979).

Dwarfism

This is certainly the most familiar clinical marker of childhood. It is widely

known that children are physically short relative to the population at large. Indeed, common clinical wisdom suggests that the treatment of the so-called “small child” (or “tot”) is particularly difficult. These children are known to exhibit infantile behavior and display a startling lack of insight (Tom & Jerry, 1967).

Emotional Lability and Immaturity

This aspect of childhood is often the only basis for a clinician’s diagnosis. As a result, many otherwise normal adults are misdiagnosed as children and must suffer the unnecessary social stigma of being labeled a “child” by professionals and friends alike.

Knowledge Deficits

While many children have IQ’s within or even above the norm, almost all will manifest knowledge deficits. Anyone who has known a real child has experienced the frustration of trying to discuss any topic that requires some general knowledge. Children seem to have little knowledge about the world they live in. Politics, art, and science—children are largely ignorant of these. Perhaps it is because of this ignorance, but the sad fact is that most children have few friends who are not, themselves, children.

Legume Anorexia

This last identifying feature is perhaps the most unexpected. Folk wisdom is supported by empirical observation—children will rarely eat their vegetables (see Popeye, 1957, for review).

Causes of Childhood

Now that we know what it is, what can we say about the causes of childhood? Recent years have seen a flurry of theory and speculation from a number of perspectives. Some of the most prominent are reviewed below.

Sociological Model

Emile Durkind was perhaps the first to speculate about sociological causes of

childhood. He points out two key observations about children: 1) the vast majority of children are unemployed, and 2) children represent one of the least educated segments of our society. In fact, it has been estimated that less than 20% of children have had more than a fourth grade education.

Clearly, children are an “out-group.” Because of their intellectual handicap, children are even denied the right to vote. From the sociologist’s perspective, treatment should be aimed at helping assimilate children into mainstream society. Unfortunately, some victims are so incapacitated by their childhood that they are simply not competent to work. One promising rehabilitation program (Spanky & Alfalfa, 1978) has trained victims of severe childhood to sell lemonade.

Biological Model

The observation that childhood is usually present from birth has led some to speculate on a biological contribution. An early investigation by Flintstone and Jetson (1939) indicated that childhood runs in families. Their survey of over 8,000 American families revealed that over half contained more than one child. Further investigation revealed that even most non-child family members had experienced childhood at some point. Cross-cultural studies (e.g., Mowgli & Din, 1950) indicate that familial childhood is even more prevalent in the Far East. For example, in Indian and Chinese families, as many as three out of four family members may have childhood.

Impressive evidence of a genetic component of childhood comes from a large-scale twin study by Brady and Partridge (1972). These authors studied over 106 pairs of twins, looking at concordance rates for childhood. Among identical or monozygotic twins, concordance was unusually high (.92), i.e., when one twin was diagnosed with childhood, the other twin was almost always a child as well.

Psychological Models

A considerable number of psychologically-based theories of the development of childhood exist. They are too numerous to review here. Among the more familiar models are Seligman’s “learned childishness” model. According to this model, individuals who are treated like children eventually give up and

become children. As a counterpoint to such theories, some experts have claimed that childhood does not really exist. Szasz (1980) has called “childhood” an expedient label. In seeking conformity, we handicap those whom we find unruly or too short to deal with by labeling them “children.”

Treatment of Childhood

Efforts to treat childhood are as old as the syndrome itself. Only in modern times, however, have humane and systematic treatment protocols been applied. In part, this increased attention to the problem may be due to the sheer number of individuals suffering from childhood. Government statistics (DHHS) reveal that there are more children alive today than at any time in our history. To paraphrase P.T. Barnum: “There’s a child born every minute.”

The overwhelming number of children has made government intervention inevitable. The nineteenth century saw the institution of what remains the largest single program for the treatment of childhood—so-called “public schools.” Under this colossal program, individuals are placed into treatment groups based on the severity of their condition. For example, those most severely afflicted may be placed in a “kindergarten” program. Patients at this level are typically short, unruly, emotionally immature, and intellectually deficient. Given this type of individual, therapy is of necessity very basic. The strategy is essentially one of patient management and of helping the child master basic skills (e.g., finger-painting).

Unfortunately, the “school” system has been largely ineffective. Not only is the program a massive tax burden, but it has failed even to slow down the rising incidence of childhood.

Faced with this failure and the growing epidemic of childhood, mental health professionals are devoting increasing attention to the treatment of childhood. Given a theoretical framework by Freud’s landmark treatises on childhood, child psychiatrists and psychologists claimed great successes in their clinical interventions.

By the 1950s, however, the clinicians’ optimism had waned. Even after years of costly analysis, many victims remained children. The following case (taken from Gumbie & Pokey, 1957) is typical.

Billy J., age 8, was brought to treatment by his parents. Billy's affliction was painfully obvious. He stood only 4'3" high and weighed a scant 70 pounds, despite the fact that he ate voraciously. Billy presented a variety of troubling symptoms. His voice was noticeably high for a man. He displayed legume anorexia and, according to his parents, often refused to bathe. His intellectual functioning was also below normal—he had little general knowledge and could barely write a structured sentence. Social skills were also deficient. He often spoke inappropriately and exhibited “whining behavior.” His sexual experience was non-existent. Indeed, Billy considered women “icky.”

His parents reported that his condition had been present from birth, improving gradually after he was placed in a school at age 5. The diagnosis was “primary childhood.” After years of painstaking treatment, Billy improved gradually. At age 11, his height and weight have increased, his social skills are broader, and he is now functional enough to hold down a “paper route.”

After years of this kind of frustration, startling new evidence has come to light which suggests that the prognosis in cases of childhood may not be all gloom. A critical review by Fudd (1972) noted that studies of the childhood syndrome tend to lack careful follow-up. Acting on this observation, Moe, Larrie, and Kirly (1974) began a large-scale longitudinal study. These investigators studied two groups. The first group comprised 34 children currently engaged in a long-term conventional treatment program. The second was a group of 42 children receiving no treatment. All subjects had been diagnosed as children at least 4 years previously, with a mean duration of childhood of 6.4 years.

At the end of one year, the results confirmed the clinical wisdom that childhood is a refractory disorder—virtually all symptoms persisted and the treatment group was only slightly better off than the controls.

The results, however, of a careful 10-year follow-up were startling. The investigators (Moe, Larrie, Kirly, & Shemp, 1984) assessed the original cohort on a variety of measures. General knowledge and emotional maturity were assessed with standard measures. Height was assessed by the “metric system” (see Ruler, 1923), and legume appetite by the Vegetable Appetite Test (VAT) designed by Popeye (1968). Moe *et al.* found that subjects

improved uniformly on all measures. Indeed, in most cases, the subjects appeared to be symptom-free. Moe *et al.* report a spontaneous remission rate of 95%, a finding which is certain to revolutionize the clinical approach to childhood.

These recent results suggest that the prognosis for victims of childhood may not be so bad as we have feared. We must not, however, become too complacent. Despite its apparently high spontaneous remission rate, childhood remains one of the most serious and rapidly growing disorders facing mental health professionals today. And, beyond the psychological pain it brings, childhood has recently been linked to a number of physical disorders. Twenty years ago, Howdi, Doodi, and Beauzeau (1965) demonstrated a six-fold increased risk of chicken pox, measles, and mumps among children as compared with normal controls. Later, Barby and Kenn (1971) linked childhood to an elevated risk of accidents—compared with normal adults, victims of childhood were much more likely to scrape their knees, lose their teeth, and fall off their bikes.

Clearly, much more research is needed before we can give any real hope to the millions of victims wracked by this insidious disorder.

References

- American Psychiatric Association (1985). The diagnostic and statistical manual of mental disorders, 4th edition: A preliminary report, Washington, D.C.: APA.
- Barby, B., & Kenn, K (1971). The plasticity of behavior. In B. Barby & K Kenn (Eds.), *Psychotherapies R Us*. Detroit: Ronco Press.
- Brady, C, & Partridge, S. (1972). My dad's bigger than your dad. *Acta Eur. Age*, 9, 123–126.
- Flintstone, F., & Jetson, G. (1939). Cognitive mediation of labor disputes. *Industrial Psychology Today*, 2, 23-35.
- Fudd, E. J. (1972). Locus of control and shoe-size. *Journal of Footwear Psychology*, 78, 345–356.

Gumbie, G., & Pokey, P. (1957). A cognitive theory of iron smelting. *Journal of Abnormal Metallurgy*, 45, 235–239.

Howdi, C., Doodi, C., & Beauzeau, C. (1965). Western civilization: A review of the literature. *Reader's Digest*, 60, 23–25.

Moe, R., Larrie, T., & Kirly, Q. (1974). State childhood vs. trait childhood. *TV Guide*, May 12-19, 1–3.

Moe, R., Larrie, T., Kirly, Q., & Shemp, C. (1984). Spontaneous remission of childhood. In W. C. Fields (Ed.), *New hope for children and animals*. Hollywood: Acme Press.

Popeye, T. S. M. (1957). The use of spinach in extreme circumstances. *Journal of Vegetable Science*, 58, 530–538.

Popeye, T. S. M. (1968). Spinach: A phenomenological perspective. *Existential Botany*, 35, 908–913.

Rogers, F. (1979). *Becoming my neighbor*. New York: Soft Press.

Ruler, Y. (1923). Assessing measurement protocols by the multi-method multiple regression index for the psychometric analysis of factorial interaction. *Annals of Boredom*, 67, 1190–1260.

Spanky, D. & Alfalfa, Q. (1978). Coping with puberty. *Sears Catalogue*, 45–46.

Suess, D. R. (1983). A psychometric analysis of green eggs with and without ham. *Journal of Clinical Cuisine*, 245, 567–578.

Temple-Black, S. (1982). Childhood: An ever-so sad disorder. *Journal of Precocity*, 3, 129–134.

Tom, C., & Jerry, M. (1967). Human behavior as a model for understanding the rat. In M. de Sade (Ed.), *The rewards of punishment*. Paris: Bench Press.

Further Readings

Christ, J. H. (1980). Grandiosity in children. *Journal of Applied Theology*, 1, 1–1000.

Joe, G. I. (1965). Aggressive fantasy as wish fulfillment. *Archives of General MacArthur*, 5, 23–45.

Leary, T. (1969). Pharmacotherapy for childhood. *Annals of Astrological Science*, 67, 456–459.

Kissoff, K. G. B. (1975). Extinction of learned behavior. Paper presented to the Siberian Psychological Association, 38th Annual Meeting, Kamchatka.

Smythe, C., & Barnes, T. (1979). Behavior therapy prevents tooth decay. *Journal of Behavioral Orthodontics*, 5, 79–89.

Potash, S., & Hoser, B. (1980). A failure to replicate the results of Smythe and Barnes. *Journal of Dental Psychiatry*, 34, 678–680.

Smythe, G., & Barnes, T. (1980). Your study was poorly done: A reply to Potash and Hoser. *Annual Review of Aquatic Psychiatry*, 10, 123–156.

Potash, S., & Hoser, B. (1981). Your mother wears army boots: A further reply to Smythe and Barnes. *Archives of Invective Research*, 56, 570–578.

Smythe, C., & Barnes, T. (1982). Embarrassing moments in the sex lives of Potash and Hoser: A further reply. *National Enquirer*, May 16.

[image file=image_rsrc5H7.jpg] [image file=image_rsrc5H8.jpg] 1 The author would like to thank all the little people.2 This research was funded in part by a grant from Bazooka Gum.Psychotherapy of the Dead

Samuel E. Menahem, Ph.D.

It is time to “bury the myth” that certain people are untreatable by modern psychotherapy. In recent years people with untreatable “narcissistic character disorders” have suddenly become treatable. It is the contention of this author that there is one group that has been totally neglected by psychotherapists—the dead. Why have they been so ignored? Probably because fat cat therapists only want to take on articulate, motivated patients. Well, it’s time for these

lazy doctors to get off their dimpled derrieres and “break new ground.” People who are dying to get into treatment can no longer be ignored. The author is now treating dead patients and training young therapists to do the same.

Review of the Literature

An exhaustive review of the psychological literature turned up only one article related to the subject of therapy of the dead. Dr. I. M. Bananas (1916) reported that death was the crucial turning point in his treatment of Frau Rigormortis, an arrogant, vindictive patient. Dr. Bananas did a five-year follow-up with the patient’s family. They reported that there were no incidents of arrogant behavior since the patient’s death. Thus, the case was considered a complete cure.

Dead Silence

Many beginning psychotherapists have trouble with long silences during the therapeutic hour. The author has found that one of the best ways to desensitize psychologists in training is to assign them a dead patient. Techniques to deal with the silence include: